

Adrabetadex Provider Net-Cost Recovery Summary																									
Client-ready economics readout for selected launch scenario																									
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Per-dose economics																									
Drug acquisition per dose		\$29,991.00																							
Allowed drug reimbursement per dose		\$40,170.00																							
Drug spread per dose		\$10,179.00																							
Procedure revenue per dose		\$500.00																							
Non-drug operating cost per dose		\$1,700.00																							
Denial drag per dose		\$950.73																							
Financing drag per dose		\$493.00																							
Net operating margin per dose		\$7,535.27																							
Break-even support per dose		\$0.00																							
Annual economics																									
Annual administrations		\$26																							
Annual drug acquisition		\$779,766																							
Annual allowed drug reimbursement		\$1,044,420																							
Annual non-drug operating cost		\$44,200																							
Annual net operating margin		\$195,917																							
Annual break-even support		\$0																							
Cash exposure		\$179,946																							
Annual 340B acquisition advantage		\$234,234																							
Margin, cash, and operational risk																									
Margin risk	Low margin risk	Per-dose margin has stronger cushion under modeled assumptions.																							
Cash exposure risk	Moderate cash exposure	Monitor outstanding administrations and remittance timing.																							
Operational risk	Lower operational risk	Operational control posture is stable.																							
Where the value is																									
340B acquisition lowers per-dose acquisition cost materially versus WAC.																									
Pass-through timing improves launch-bridge payment visibility and shortens initial uncertainty.																									
Clean PA, NDC, billing, wastage, and 340B documentation remain critical to realizing any margin.																									
Scenario interpretation																									
Primary value is strongest in the 340B center-of-excellence / HOPD launch path; non-340B cases should remain clearly.																									
Permanent HCPCS timing is still important for long-horizon clarity, but it does not replace launch bridge controls.																									
Keep operational requirements explicit: PA status, NDC reporting, invoice support, and claims edits quality.																									
Estimates are for planning purposes only and do not guarantee coverage, coding, payment, or reimbursement. Providers should validate payer-specific rules, payer edits, and operational controls before claim submission.																									